



Steroids in Upper Respiratory Infections (URIs)

For uncomplicated URIs, corticosteroids do not significantly reduce the duration or severity of symptoms and may have potential side effects.

Side effects of systemic corticosteroids include increased risk of infection, elevated blood sugar, weight gain and mood changes.

Long term side effects include osteoporosis, glaucoma, cataracts, muscle weakness and wasting.

Systemic corticosteroids may be indicated in upper respiratory infections when there is significant inflammation present, such as in cases of **acute exacerbations** of chronic obstructive pulmonary disease (COPD) or in cases of moderate to severe asthma exacerbations. Maintenance therapy is with inhaled glucocorticoids.

Systemic corticosteroids are also indicated if there are signs of significant airway swelling or severe pain, such as in severe tonsillitis, peritonsillar abscess, croup, epiglottitis.

Pharyngitis (viral or bacterial)

- Do not prescribe oral corticosteroids routinely for pain.
- Only given to patients with severe throat pain and extreme difficulty swallowing (1-2 day course)
- Benefit of slightly decreasing the duration of throat pain

Acute rhinosinusitis (Sinusitis)

- Oral corticosteroids are not indicated; may shorten time to symptom resolution but benefits are small.
- Consider the following treatments:
 - OTC NSAIDs, acetaminophen, saline irrigation, intranasal corticosteroids, intranasal saline spray, intranasal ipratropium bromide, oral decongestants, intranasal decongestants (no more than 3 days), antihistamines

Croup

- Mild croup (no stridor at rest, no high risk features and no high risk medical conditions)
 - Single dose dexamethasone or prednisolone
- Moderate croup - (stridor at rest) should be evaluated in the ER

Bronchiolitis

- Younger children: Supportive care is the mainstay of treatment. Use of humidifier and steamy showers can be helpful. Inhaled bronchodilators, inhaled or systemic steroids are not routinely recommended.

Bronchospasm

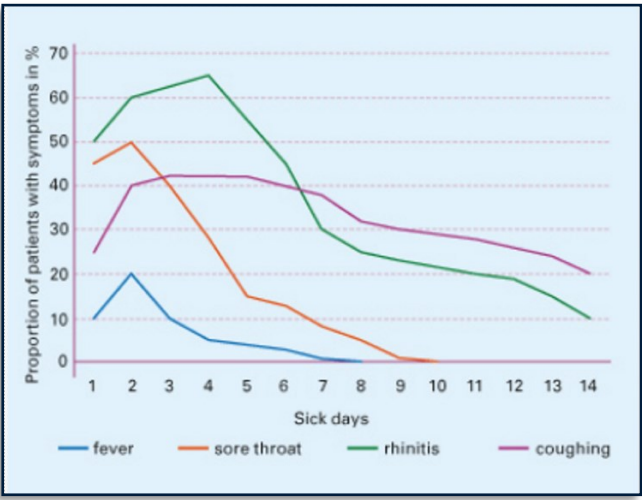
- Children and adults with asthma: RSV and other viruses can cause bronchial reactivity - oral corticosteroids and bronchodilators may be beneficial.

Bronchitis

- Corticosteroids have not been shown to reduce severity or duration of cough in adults with acute bronchitis.

Temporal course of respiratory tract infection symptoms

Cough and rhinitis may last beyond 2 weeks in some patients, so let's manage expectations.



The Power of the Search Bar

On **RVMC Clin Ops SharePoint** - can search for GCN refresher video, Telederm Job Aid, Strep testing, etc.



On **Teams** - can search previous posts, questions, answers and workflows - especially useful during after hours when there are no mentors on.



Special thanks to Dr. Adetunji Adegboyega for reminding us about the power of the search bar.

REMINDER: Please note the VMC break policy is 10 minutes for a consecutive 4 hour shift. For an 8 hour shift, you can choose to take a 20 minute break or two 10 minute breaks. Do not take these breaks at the beginning or end of your shift.